

Sell the room in somebody’s day.

The platform is built and largely proven. It has no customers. This is the plan to get the first hundred — in one city, through the people who already gather them, and who cannot be paid to recommend you.

Launch city Greater Manchester Motion Referral-led, community-first Horizon 90 days

Committed budget £16,940 Lean floor £2,760

01	The honest starting position
02	Positioning, and the one sentence
03	Customer segments, ranked
04	The launch city, locked
05	The first 100 customers
06	The 30 / 60 / 90

07	Suppliers, and how to source them
08	The budget, line by line
09	Acquisition economics
10	Marketing engine
11	Metrics and kill thresholds
12	Risks, and this week

01 The honest starting position

What is true today, separated from what merely exists.

A go-to-market plan built on an inflated view of readiness fails in week three, when the demo breaks in front of the buyer who said yes. So this begins with the ledger.

0 Customers acquired to date	773 Automated tests passing	£5.99 Premium, per month, on the web	£2 Organisation floor, per seat
10–100 Age range served			

THE CAUSE OF ZERO, FOUND AND FIXED

Registration worked perfectly and returned 201 every time it was tested. It was simply unreachable: eleven marketing pages, and not one had a primary call to action that arrived at an account. The nav pointed at an onboarding explainer that ended in “Request access” pointing at a `mailto:` form.

This matters more than any channel choice. It means the absence of customers has never been evidence about demand — the experiment was never run. Every number in this document is a first measurement, not a recovery.

READINESS LEDGER

CAPABILITY	STATE	WHAT THAT MEANS FOR SELLING
Registration reachable from every page	PROVEN	Build-time check fails if it regresses. Safe to drive traffic.
Falls / strength & balance module	PROVEN	The wedge for the highest-intent channel.
Assurance page & clinical hazard log	PROVEN	An NHS or school buyer can diligence you without a meeting.
Referral routes for care settings	PROVEN	Nine channel codes, no commission attached.
Funnel instrumentation	PROVEN	Five steps, visible drop-off. You will know which screen kills it.
Weekly newsletter, consent-gated	PROVEN	Retention engine. Currently 5 eligible recipients.
Blog engine + editorial review gate	PROVEN	Organic surface. Needs a named human reviewer weekly.
PWA install + launch screens	PROVEN	No app-store commission: £5.99, not £8.99.
Email delivery (SMTP credentials)	BLOCKED	Nothing sends. Renders and logs as sandbox. Fix before day 1.

CAPABILITY	STATE	WHAT THAT MEANS FOR SELLING
AI provider keys verified working	UNVERIFIED	Probe button exists, never run against production.
DCB0129 clinical safety sign-off	BLOCKED	Needs a named Clinical Safety Officer. Gates NHS revenue.
Stripe live keys & price IDs	UNVERIFIED	No confirmed end-to-end payment. Gates every pound.

FOUR THINGS GATE REVENUE, AND NONE IS A MARKETING PROBLEM

SMTP credentials, a live Stripe path, working AI keys, and a Clinical Safety Officer. Until those are closed, acquisition spend converts interest into refunds and apologies. Days 1–14 exist to close them.

02 Positioning, and the one sentence

What you are, to whom, against what alternative.

Every fitness app sells transformation. That is a crowded, expensive, low-trust category where the incumbent is a competitor's brand budget and the customer's own failed January. Jess Move should not enter it.

The defensible position is the opposite claim: this is not a fitness product, it is a product about the day you already have. Two minutes. Between things. Ages ten to a hundred. That framing wins in exactly the places a fitness app is refused — a falls group, a care home, a school, a link worker's caseload.

THE SENTENCE

Jess Move finds the movement your day already has room for — two minutes at a time, for anyone from ten to a hundred — and it never tells you you're fine when you're not.

TO A CARE PROFESSIONAL

It gives someone a starting level, not a risk score. It says out loud that a good result isn't evidence of low risk. And nobody is paid to recommend it.

TO AN EMPLOYER

Group reporting with a floor of eight people. You see whether a programme is working. You never see a person.

THE THREE CLAIMS THAT DO THE WORK

Claim 01 – refusals

It says what it will not do

A published hazard log, enforced refusals, and a strength module that states a good score is not evidence of low risk. In health, the product that under-claims is the one a professional can pass on.

Claim 02 – no conflict

Nobody is paid to refer

No commission, no per-signup fee, no revenue share. This is the single most valuable asset you own for clinical and public-sector channels, where a fee is a disqualifying conflict of interest.

Claim 03 – the household

Ten to a hundred, one account

Family at £12.99 covers four people across child and later-life modes with guardian controls. No competitor in falls prevention also serves a ten-year-old.

What not to say

Never lead with the AI

No vendor or model names on public pages — a standing rule, and correct. Buyers in this market hear “AI” as risk. Lead with the refusals; the intelligence is how, not what.

03 Customer segments, ranked by reachability

Not by size. Size without a route is a daydream.

Segments are ranked by cost to reach the first paying account, because with zero customers your scarcest resource is proof, not addressable market.

RANK	SEGMENT	BUYS	PRICE	ROUTE IN	CYCLE
1	Falls-prevention course graduates	Premium / Family	£5.99–12.99	Instructor hands it over at the last session	Days
2	Social prescribing caseloads	Premium	£5.99	Link worker, at the end of a six-week block	1–3 wks
3	Retirement & sheltered housing	Organisation	£2–5/seat	Scheme manager, activities coordinator	4–8 wks
4	Community pharmacy	Premium / Family	£5.99–12.99	Counter conversation, medication review	1–2 wks
5	Adult children buying for a parent	Family	£12.99	Organic search, the falls content cluster	Days
6	Care providers	Organisation	£2–5/seat	Registered manager; CQC evidence angle	6–12 wks

RANK	SEGMENT	BUYS	PRICE	ROUTE IN	CYCLE
7	SME employers, 50–500 staff	Organisation	£2–5/seat	HR / wellbeing lead, sedentary-desk angle	8–16 wks
8	Schools	Organisation	£2–5/seat	PE or wellbeing lead; safeguarding-first	A term
9	GP practices & PCNs	Organisation	£2–5/seat	Blocked until DCB0129 is signed	3–9 mo

THE STRATEGIC READ

Ranks 1, 2 and 4 are consumer revenue reached through an institutional door. That is the whole trick: the trust and concentration of a B2B channel, while selling a £5.99 consumer subscription that needs no procurement, no contract, no information-governance review and no Clinical Safety Officer.

Ranks 3, 6, 7 and 8 are contracts worth ten to a hundred times more each — and they take a quarter to close. Start them in month one, expect them in month four.

04 **The launch city, locked**

One city. Everything else is a distraction until it repeats.

A referral motion is a travel motion. Every room in this plan is a room you walk into, which means the launch city is not a market-sizing decision — it is a decision about how many rooms you can physically reach in a week.

LOCKED: GREATER MANCHESTER

Manchester is the UK’s first age-friendly city, admitted to the WHO Global Network for Age-Friendly Cities in June 2010, with a dedicated older people’s programme running since 2003 and a current strategy, *Manchester: A city for life 2023–28*. Greater Manchester became the first age-friendly *city region* in the UK, and in February 2025 launched a case-finding falls-prevention pilot supported by the Centre for Ageing Better.

That matters for one specific reason: the rooms already exist and are already funded. You are not creating demand for organised movement among older people in Manchester — a public programme has spent two decades creating it. You are arriving at the moment those programmes end, which is the gap nobody funds.

WHY MANCHESTER BEATS THE ALTERNATIVES

CITY	FALLS / AGEING INFRASTRUCTURE	COMPACTNESS	VERDICT
Greater Manchester	UK’s first age-friendly city and first age-friendly city region; active falls-prevention pilot; single integrated care system	Ten boroughs on one tram and rail network — three rooms reachable in a day	LOCKED
Birmingham	Large older population; council under financial distress, commissioned activity cut	Sprawling, car-dependent	SECOND
Leeds / Sheffield	Strong voluntary sector, good social prescribing	Compact centres, thinner falls-specific programmes	THIRD
London	Largest market, most services	Travel times destroy the room-per-day maths; highest competition for a professional’s attention	AVOID FIRST

THE ONE ASSUMPTION BEHIND THIS LOCK

That you can be in Greater Manchester most weeks. This is the only input to the decision I do not have. If you are based elsewhere, the answer changes to your own city immediately — a founder two hours from the rooms will book three a month instead of three a week, and the plan fails on travel rather than on product.

The selection criteria are what matter, and they are portable: a funded ageing or falls programme, a single integrated care system, a public transport network that puts three rooms within a day, and housing providers with local stock. Apply them to where you live and relock.

THE NAMED TARGET LIST — FIRST SIX APPROACHES

TARGET TYPE	WHERE TO FIND THE SPECIFIC NAMES	APPROACH BY
Age-Friendly Manchester network	Manchester City Council's Age-Friendly team publishes its neighbourhood networks and older people's forums	Day 7
Commissioned strength & balance providers	GM local authority public health teams publish commissioned providers; Later Life Training holds the instructor register	Day 7
Social prescribing link workers	Every PCN in GM employs them; route via GM Integrated Care and local CVS infrastructure bodies	Day 10
Sheltered / retirement housing	Anchor, Housing 21 and ExtraCare all hold stock across GM; also GM housing providers via the local authority register	Day 14
Independent community pharmacy	NHS England pharmacy directory filtered by GM postcodes — independents, not multiples	Day 14
Care providers	CQC register filtered to GM, then filtered to “requires improvement” on responsive or effective	Day 21

05 The first 100 customers

Counted properly — a registration and a payment are not the same event.

A CORRECTION TO THE EARLIER VERSION OF THIS PLAN

The first draft counted “accounts” as though a person leaving a room became a paying customer. They do not. Free has no card and lasts forever; the AI allowance is 50 ACU a month for two months. So a room produces *registrations*, and a separate conversion produces *payments* — and 100 paying accounts needs roughly 45 rooms, not 17.

TWO SCENARIOS, HONESTLY SEPARATED

SCENARIO	ROOMS	REGISTRATIONS	FREE→PAID	PAYING	MRR AT DAY 90
Base — part-time, ~2 rooms/wk	20	128	28%	36	£291
Base — conversion holds	20	128	35%	45	£364
Committed — full-time, ~4 rooms/wk	45	278	28%	78	£631
Committed — the 100 target	45	278	35%	97	£785

Assumptions: 15 people per room, 32% take-up on a warm in-person handover, plus organic search and word of mouth at roughly 15% of room volume. Blended price of £7.09 — 70% Premium at £5.99, 30% Family at £12.99.

READ THE LAST COLUMN BEFORE YOU CELEBRATE THE FOURTH

A hundred paying customers is £785 a month. That is not a business, and pretending otherwise is how founders raise on the wrong story. What ninety days buys is not revenue — it is a proven, repeatable room: a named contact, a known take-up rate, and a conversion number you measured rather than modelled.

That asset is what makes month four to twelve possible, and what makes an organisation contract or an investor conversation defensible. Sell the mechanism, not the MRR.

WHERE THE ROOMS COME FROM

ROOM TYPE	ROOMS (COMMITTED)	WHY THEY CONVERT
Falls-prevention course endings	16	Course ends; the fear of losing progress is live and unaddressed
Social prescribing block endings	10	Same moment, delivered by a trusted professional
Sheltered housing activity groups	8	Social proof in the room; shared challenges work immediately
Community pharmacy sessions	7	Medication review already raises weight, appetite and mobility
Older people's forums & networks	4	Age-Friendly Manchester convenes these; one visit reaches several groups
Total	45	

THE ASK — TO A FALLS GROUP INSTRUCTOR, VERBATIM

“Your course ends and people lose what they built — that’s the bit nobody funds. I’ve built something for exactly that week. I’m not asking you to recommend it. I’m asking to sit in on your last session, and if it’s any good you can decide afterwards. Nobody gets paid for this, including you — there’s no commission behind it, deliberately.”

THE CLOSE

“It’s free, there’s no card, and it stays free. If you want the coaching part it’s £5.99 a month, and you can see whether it’s worth it before you decide.”

FALSIFY BY ROOM THREE

If the first three rooms register below 15% of the people in them, the problem is the handover moment or the offer — and no number of extra rooms fixes it. Stop and change the offer before booking room four. This costs three weeks and about £60 of travel to learn.

06 The 30 / 60 / 90

Each phase has one job and one gate. Do not start the next until the gate passes.

DAY 1 – 30

Make it possible to pay you

- Days 1–7 · Unblock. SMTP credentials in Vercel. Stripe live keys and price IDs. Run the AI provider probe. Confirm the deploy is past the branch head.
 - Days 3–10 · Buy your own product. From a phone, as a stranger, on mobile data. Pay real money. Cancel. Refund yourself.
 - Days 5–14 · Appoint a Clinical Safety Officer. Longest lead time of anything here.
 - Days 7–21 · Book eight rooms across Greater Manchester. Falls groups and link workers only.
 - Days 10–30 · Run six. Observe, do not pitch. Record take-up per room.
 - Ongoing · Approve one newsletter and two articles weekly. Ninety minutes.
-

GATE TO PHASE 2

Money settled in the bank from a stranger. Six rooms run. Registration take-up above 15% and a funnel that shows where the rest stopped.

DAY 31 – 60

Prove the room repeats

- Repeat the winning room type. Whichever of the six converted best, book twelve more of exactly that.
 - Fix the worst funnel step. You will have real drop-off data; one screen will own most of the loss.
 - Open two organisation conversations. Sheltered housing and one care provider. Expect nothing before day 90.
 - Publish the falls content cluster. Twelve articles aimed at what an adult child types at 11pm.
 - Ask your first twenty registrants: “What nearly stopped you?”
 - Watch the two-month ACU cliff hit your earliest cohort. This is your first real conversion signal.
-

GATE TO PHASE 3

Twenty rooms run. Free-to-paid conversion measured, not modelled. Month-one retention above 60%. One organisation proposal on the table.

Turn one room into a channel

- Recruit five standing referrers — people who hand it on every cohort, not once.
- Close one organisation contract. Ten seats minimum, annual. Any logo beats a bigger pipeline.
- Build the referrer pack. One page a professional can hand over without explaining anything.
- Instrument every code. Nine referrer kinds already exist — give each room type its own.
- Decide on app stores. £8.99 versus £5.99 is a 33% margin question, not a distribution one.
- Write the case study. One named Manchester group, real numbers, reviewed by them.

GATE TO SCALING SPEND

45 rooms run. Payback under 60 days, measured. A repeatable room type with a named contact who will do it again unprompted.

WHAT IS DELIBERATELY ABSENT FROM ALL NINETY DAYS

No paid advertising. No app-store launch. No press. No conference stand. No influencer. Each spends money to find people you cannot yet keep, before you know which screen loses them. Paid acquisition begins on day 91 at the earliest, and only if payback under 60 days has been measured rather than assumed.

07 Suppliers, and how to source them

A software business still has a supply chain. Yours has one urgent gap.

Your standing rule is no vendor beyond Vercel and Firebase. Keep it. Every additional vendor is a data processor, a contract, a security review and a question in every enterprise sale. It means the list below is mostly *people*, not platforms.

SUPPLIER	WHY NEEDED	HOW TO SOURCE	LEAD TIME
Clinical Safety Officer CRITICAL PATH	DCB0129 requires a named, registered clinician to sign the hazard log. Without one, NHS, PCN and most care-provider revenue is closed.	A registered clinician (doctor, nurse, pharmacist or AHP) with DCB0129 training. Available as a part-time retained role via clinical safety consultancies — this is a recognised freelance appointment, not a hire.	4–10 wks
Transactional email	Nothing sends today. Gates confirmation, password reset, guardian consent and the newsletter.	Already architected for SMTP with a Hostinger default. Credentials into Vercel. No new vendor.	1 day

SUPPLIER	WHY NEEDED	HOW TO SOURCE	LEAD TIME
Payments	Stripe. Live keys, price IDs, one confirmed end-to-end payment and one tested refund.	Already integrated. Needs live-mode verification only.	2–5 days
Named editorial reviewer	No agent-written health copy may publish unreviewed. A hard control with no person currently attached.	Can be you. Must be named, weekly, ~90 minutes. If it is not diarised the content engine stalls.	Immediate
DPIA / data protection	Health data, minors, employers. Every organisation buyer asks for a DPIA and a processing agreement before signing.	Fractional DPO, or a one-off DPIA from a health-tech-literate privacy consultant. ICO registration is a legal minimum.	2–4 wks
Cyber Essentials	Frequently the actual procurement requirement, ahead of a full penetration test.	Self-assessment certified through IASME. Plus (audited) only when a contract demands it.	1–3 wks
Penetration test	Enterprise and public sector buyers ask for a recent report.	CREST-accredited tester. Do this before the first organisation contract, not during it.	3–6 wks
Accessibility audit	Public sector procurement requires WCAG 2.2 AA. Your users include ninety-year-olds — this is product quality, not paperwork.	A specialist auditor. Prioritise registration and the strength module.	2–4 wks
Insurance	Professional indemnity and cyber. Organisation contracts specify minimum cover, typically £1–5m.	A broker who understands digital health, not a comparison site.	1–2 wks

SOURCING PRINCIPLE

Before signing any supplier, ask: “Does this become a question I have to answer in every future sale?” A processor holding health data does. A pen-test report does not — it *answers* questions. Buy the second kind freely and the first kind almost never.

08 The budget, line by line

Ninety days, two tiers, and a clear statement of what the cheaper one cannot buy.

Two budgets, because the honest answer depends on one decision: whether you are opening the organisation and NHS channels in the first quarter, or proving the consumer room first and paying for compliance out of revenue.

The lean floor is what it costs to legally take money from a stranger in Manchester. The committed budget adds the compliance evidence that organisation buyers ask for before they will talk.

£2,760 Lean floor — 90 days, consumer only	£16,940 Committed — opens org & NHS channels	£373 Monthly run rate after setup	£36 Cost per room, all in
--	--	---	-------------------------------------

A. RUN THE PLATFORM — UNAVOIDABLE

LINE	BASIS	MONTHLY	90 DAYS	TIER
Vercel Pro	1 seat, hosting and builds	£16	£48	Both
Firebase	Usage-based at low volume	£10	£30	Both
Email (SMTP mailbox)	Hostinger business mailbox	£5	£15	Both
AI provider usage	Metered by ACU; ~150 active free users	£60	£180	Both
Domain & DNS	Annual, apportioned	£3	£9	Both
Stripe fees	1.5% + 20p on UK cards, ~£800 processed	£12	£36	Both
Subtotal A		£106	£318	

B. BE ALLOWED TO TRADE — UNAVOIDABLE

LINE	BASIS	90 DAYS	TIER
ICO data protection fee	Tier 1 annual fee, legally required when processing personal data	£52	Both
Professional indemnity + cyber insurance	£1m cover, annual premium	£1,400	Both
Accountant — quarterly	Bookkeeping and VAT position	£450	Both
Subtotal B		£1,902	

C. GET INTO ROOMS — THE ACTUAL GO-TO-MARKET SPEND

LINE	BASIS	UNIT	QTY	90 DAYS
Travel across Greater Manchester	Tram/rail day ticket or fuel per room visit	£9	45	£405
Referrer packs	A5 double-sided colour, 500 run	£0.24	500	£120
Room goodwill	Tea, biscuits, a raffle prize — you are a guest	£12	45	£540
Printed strength-module cards	Large-print take-home, A5 card	£0.30	400	£120
Roller banner & name badge	One-off, reused every room	£85	1	£85
Case study photography	Half-day, one named group, consented	£350	1	£350
Subtotal C				£1,620

D. OPEN THE ORGANISATION AND NHS CHANNELS — COMMITTED TIER ONLY

LINE	BASIS	90 DAYS	BUYS YOU
Clinical Safety Officer	Retained appointment plus DCB0129 hazard log sign-off	£6,000	NHS, PCN and most care-provider conversations
DPIA	One-off, health-tech-literate privacy consultant	£2,400	The document every organisation buyer requests
Cyber Essentials	IASME self-assessment certification	£360	The usual procurement floor
Accessibility audit	WCAG 2.2 AA, registration flow and strength module	£2,800	Public sector eligibility; and a better product
Subtotal D		£11,560	

TOTALS

TIER	INCLUDES	90-DAY TOTAL	WHAT IT CANNOT DO
Lean floor	A + B + a 20-room version of C (£540)	£2,760	Cannot sell to NHS, PCNs or most care providers. Cannot answer a procurement questionnaire.
Committed	A + B + C + D, plus 10% contingency (£1,540)	£16,940	—

WHAT THE BUDGET WILL NOT RETURN INSIDE 90 DAYS

At the committed target of 97 paying customers you finish the quarter at roughly £785 MRR against £16,940 spent. On a pure cash view that is twenty-two months to recover, and if you judge the quarter that way you will conclude wrongly that it failed.

Most of the £16,940 is not acquisition spend — it is one-off market access. The CSO, the DPIA, Cyber Essentials and the accessibility audit do not repeat, and they unlock a channel where a single 100-seat contract at £4 is £4,800 a year from one signature. The recurring cost of this plan is £373 a month, and the acquisition cost inside it is £1,620 for 45 rooms — £36 a room, or roughly £17 per paying customer. That number is the one to judge.

IF YOU CAN ONLY SPEND £2,760

Take the lean tier and do it properly. Twenty rooms, consumer only, no organisation conversations at all. You will finish the quarter with about 36–45 paying customers, a measured conversion rate and a repeatable room — which is exactly the evidence needed to justify the committed spend in quarter two, out of revenue or out of a raise. The one thing not to do is spend half of tier D: a DPIA with no CSO opens nothing.

All figures are UK market rates as at August 2026, exclusive of VAT, and are estimates rather than quotations. Compliance costs in particular vary by supplier and by how much evidence your existing hazard log and assurance page already provide — both are unusually complete, which should pull the CSO and DPIA figures toward the lower end.

09 Acquisition economics

What you can afford to spend, derived from what you actually charge.

Target gross margin is 50%. At £5.99 a month that is roughly £3 of gross profit per subscriber per month. Everything below follows from that, and it is unforgiving of expensive channels.

CHANNEL	CAC	PAYBACK	VERDICT AT THIS STAGE
Referral from a care setting	£17	6 mo	DO THIS Measured from the budget above.
Organic search, falls cluster	£2–10	1–4 mo	DO THIS Compounds. Slow to start.
Word of mouth / household	≈£0	Immediate	DO THIS Family plan is the mechanism.
Newsletter to existing members	≈£0	—	RETENTION Judge on churn, not signups.
Organisation contract	£300–1,500	4–9 mo	Q2 10 seats × £2 = £20/mo. Needs 50+ seats.

CHANNEL	CAC	PAYBACK	VERDICT AT THIS STAGE
Paid social	£25-60	8-20 mo	NOT YET Longer than your retention data exists.
App store launch	33% of price	—	NOT YET £8.99 vs £5.99 to a middleman.

THE ORGANISATION-PRICING TRAP

Ten seats at the £2 floor is £20 a month — less than three consumer subscribers, for a sale that takes three months and a DPIA. The minimum viable organisation deal is closer to fifty seats, or £5 per seat. Price the floor for schools and charities where the logo is worth more than the revenue; quote everyone else at £4-5 and let the contract close the gap.

THE FREE TIER, READ AS AN ACQUISITION INSTRUMENT

Free forever with no card, and 50 ACU a month of AI for two months. This is unusually well judged for this market: the people you are selling to are frequently burned by subscriptions they could not cancel, and “no card, nothing charged” removes the single biggest objection a link worker raises on a customer’s behalf.

The risk is the cliff. In month three the AI allowance stops. Instrument that moment specifically — it is your only real conversion event. If conversion at the cliff is under 20%, the fix is a smaller permanently-free AI allowance, not a longer trial.

10 Marketing engine

Four surfaces, already built. None of them is advertising.

Surface 01 — owned

The falls content cluster

Write for the adult child at 11pm, not the clinician. “Exercises to stop falling”, “my mum is scared of falling”, “what to eat on appetite-suppressing medication”. Twelve articles beats a hundred thin ones. Every article must reach registration in one click.

Surface 02 — earned

The assurance page

A published hazard log and a list of enforced refusals lets a buyer diligence you at midnight without booking a meeting. This shortens institutional sales more than any brochure. Link it in the first email of every organisation conversation.

Surface 03 – retained

The weekly newsletter

Composed from your own feature pages, so it cannot promise something that does not exist. Its job is not acquisition — it is stopping the month-two drop-off by showing people the product they already have.

Surface 04 – the multiplier

The referrer pack

One page. What it does, what it refuses to do, the privacy answer, and a code. A professional will not deliver your pitch — they will hand over a piece of paper. Make that paper excellent and the channel scales without you in the room.

ON THE AGENCY RECOMMENDATION — MARKETWAROS.COM

I could not evaluate this. The site is unreachable from this environment: the network policy refused the request outright, and a web search returns no such domain (the near matches are MarketWatch, marketwars.com and MarketWurks, none of which appear to be what you mean).

Rather than invent a recommendation: at this stage you should probably not hire an agency at all. You have no conversion data, so an agency would be optimising a funnel nobody has measured, and you would pay several thousand pounds to learn what your first three rooms tell you for free. Nothing in the budget above is allocated to one. If you engage one anyway, require named UK health or public-sector clients you can phone, a refusal to promise leads, payment tied to a defined deliverable rather than a monthly retainer, and no access to member data of any kind. Send me the correct URL and I will assess them properly.

11 Metrics, and the numbers that would end the plan

Targets are worthless without a threshold that means stop.

METRIC	DAY 30	DAY 60	DAY 90	KILL / PIVOT THRESHOLD
Rooms run	6	20	45	Cannot book 3 rooms in 21 days → the pitch is wrong
Registrations	35	125	278	—
Room take-up	28%	32%	32%	Under 15% across 3 rooms → stop booking, change the offer
Paying accounts	8	40	97	Under 10 by day 45 → the offer is wrong, not the channel
MRR	£57	£284	£785	—
Registration completion	40%	55%	65%	Under 30% → a screen problem; fix before more traffic
Month-1 retention	—	60%	70%	Under 40% → do not spend a pound on acquisition
Free-to-paid at the ACU cliff	—	—	35%	Under 10% → restructure the free allowance
Standing referrers	1	3	5	Zero repeat referrers by day 90 → it is a favour, not a channel
Cost per paying customer	—	—	£17	Above £35 → the room model does not scale

THE ONE NUMBER THAT MATTERS MOST

Room take-up. Everything else is downstream. If a trusted professional hands your product to fifteen people at the exact moment they need it and fewer than two register, no channel, no article and no advertisement will save the plan — and you will have learned it in three weeks for about £60 of travel.

12 Risks, and this week

RISK	CONSEQUENCE	MITIGATION
A safety incident attributed to the product	Existential. Ends the referral channel nationally, not locally.	The refusals are built and enforced. Never weaken them for conversion. Appoint the CSO. Never let an agent-written health claim publish unreviewed.
Payment path broken at launch	Every acquired customer lost at the last step, and you blame the channel.	Buy your own product with real money in week one. Refund yourself.
Email silently not sending	No confirmations, no resets, no newsletter. Looks like churn; is plumbing.	SMTP credentials before any room is booked. Check the mail log after room one.

RISK	CONSEQUENCE	MITIGATION
Referral treated as a favour, not a channel	Fifteen customers, then it stops, and it never repeats.	The measure is <i>standing</i> referrers who hand it on every cohort. Build the pack so it survives without you in the room.
The ACU cliff churns the first cohort	You acquire 278 and keep 30, and mistake it for a marketing problem.	Instrument the cliff now. Decide the permanent free allowance before month three arrives.
Organisation deals consume the quarter	Zero revenue and a full calendar. The classic early B2B trap.	Cap organisation effort at one day a week until a consumer room repeats.
Building instead of selling	The most likely failure by a distance. 773 tests pass and nobody has paid you.	Feature work frozen for 30 days except the four revenue blockers. Everything else waits for a customer to ask.

THIS WEEK

- Put SMTP credentials in Vercel and send yourself a real email.
- Confirm Stripe is live and buy your own subscription from a phone.
- Press the provider probe in the editorial queue and read the result.
- Register with the ICO — £52, and legally required already.
- Email six Greater Manchester falls instructors. Not commissioners. Instructors.
- Start the Clinical Safety Officer search. Longest lead time of anything you will do.
- Diarise ninety minutes on Friday to approve the newsletter and two articles.
- Write down today's funnel numbers so that in four weeks you can tell whether any of this worked.

The product problem is largely solved. The distribution problem has not been attempted once. One good room in Manchester is worth more than another month of engineering, and you will know inside three weeks whether the rooms convert — which is a far better position than yesterday, when the number was zero and the reason was unknown.

Prices, plan structures, free-tier terms, referrer channel types and the 50% gross-margin target are read from the platform's own configuration. Take-up rates, conversion rates, CAC and supplier costs are modelled estimates at UK market rates as at August 2026, exclusive of VAT — not quotations and not measurements. The platform has no conversion history yet, and every estimate here is written to be falsified inside ninety days.